

April 5, 2026

To

Co-Editor-in-Chief

Dr. Jair Soares

Journal of Affective Disorders Reports

Subject: Submission of Revised Manuscript

Dear Editor,

Thank you for the opportunity to submit a revised version of our manuscript entitled:
**“Decision-Making Autonomy and Depressive Symptoms Among Women in Lesotho:
Rural–Urban Differences.”**

We sincerely appreciate the constructive and thoughtful comments provided by the reviewers and the editorial team. In response, we have carefully revised the manuscript and prepared both a cleaned and a tracked-changes version to ensure full transparency. Specifically, we have:

- Polished the manuscript for clarity, coherence, and academic rigor.
- Corrected all typographical, spacing, and formatting inconsistencies noted by the reviewers.
- Standardized terminology and improved the presentation of confidence intervals and statistical reporting.
- Revised the Discussion section to enhance interpretive depth, link findings to theory, and align with the study objectives.
- Clarified spatial findings, strengthened conceptual explanations, and improved the logical flow of key sections.
- Updated figures, tables, and references to comply with journal standards.

Additionally, we have included a new co-author to support major revisions and manuscript improvement:

Maria Bintey Kabir

- Shaheed Suhrawardy Medical College, Dhaka-1207, Bangladesh
- Study Physician, Projahnmo Research Foundation, Dhaka-1213, Bangladesh

We are submitting the following files for your consideration:

1. **Miah_Manuscript_2026_Cleaned.docx** – clean revised version
2. **Miah_Manuscript_2026_Revised.docx** – tracked-changes version

3. **Response_Letter.pdf** – point-by-point responses to all reviewer comments with page and line references.

We hope that these revisions adequately address all concerns and further strengthen the clarity and scientific contribution of the manuscript. We are grateful for the reviewers' guidance and look forward to your consideration of the revised version.

Best regards,
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Reviewer 1

Reviewer 1 – Major Concerns and Author Responses

Comment 1:

“What is the depression situation in Lesotho? This is missing in the background section.”

Author Response [Page no: 5, Line no: 92-97]

Thank you for this important observation. We have now expanded the background section to clearly describe the burden of depression in Lesotho. Drawing on evidence from DHS Program (2023) and Jejaw et al. (2026), we added that depressive symptoms remain a major public-health concern in Lesotho, with a substantial burden of untreated mental-health conditions among women. Furthermore, based on Myint et al. (2025a), we highlight that Lesotho has one of the highest estimated depressions prevalences in Southern Africa, with women disproportionately affected. These details have now been incorporated into the revised background section.

Comment 2:

“Similarly, what is the state of autonomy in Lesotho? This detail is not provided in the background section.”

Author Response [Page no: 5-line no: 100-103]

We appreciate this constructive comment. To address it, we incorporated additional literature that documents the state of women's autonomy in Lesotho. Findings from Gebeyehu et al. (2022), Hormazábal-Salgado et al. (2024), and Viens et al. (2016) have been added to explain that women's autonomy remains notably limited—particularly in decisions related to health care, household purchases, and mobility. These studies also show that rural women experience even lower decision-making power due to entrenched gender norms and economic dependence. This contextual information has been added to strengthen the background section.

Comment 3:

“Why women, not men or both? This justification is needed in the background section.”

Author Response [Page 5, Line no:98-99]

Thank you for raising this important point. We have now provided a clearer justification for focusing on women. Drawing on evidence from DHS Program (2023), Myint et al. (2025a), Fernández et al. (2025), and Okyere et al. (2025a), we explain that depression disproportionately affects women in Lesotho, and they exhibit slightly higher help-seeking prevalence (17.1%) compared to men (16.4%). Studies further indicate that women—particularly those in urban areas—are more likely to seek help for depressive symptoms. Combined with limited autonomy and structural gender inequalities, women represent a uniquely vulnerable population requiring focused study. Additionally, autonomy measures in DHS are validated only for women, not men. This rationale has now been clearly integrated into the background section.

Comment 4:

“What is the theoretical basis for this study? Theoretically, what mechanisms does autonomy influence depression?”

Author Response (Page 6, Line 115–121):

Thank you for this insightful comment. We have now added a brief theoretical explanation grounded in **Self-Determination Theory** and **Gender and Power Theory**. These frameworks explain that limited autonomy restricts women's ability to make decisions about their health, mobility, and resources, which increases psychological stress, reduces coping capacity, and heightens vulnerability to depressive symptoms. We have incorporated these mechanisms into the background section to clarify the theoretical basis of our study. The theoretical linkage has now been added to the revised manuscript.

Reviewer Comment 5 :

“What sensitivity analysis was performed given that the PHQ-9 was used?”

Author Response (Page 12, Line 254–255):

Thank you for raising this important point. In the current study, we did not perform additional sensitivity analyses specifically for the PHQ-9 scoring. This is because we followed the standard DHS scoring protocol, which applies the validated PHQ-9 cut-off for moderate-to-severe depressive symptoms. However, we acknowledge that alternative cut-offs or item-level adjustments could provide additional robustness. We have now added a clarification in the Methods section to explain this and noted the lack of sensitivity analysis as a limitation. We appreciate the reviewer’s insight, which will inform future work.

Reviewer Comment 6 :

“The description of the exposure needs much clarification.”

Author Response (Page 8–9, Line 160–176):

We thank the reviewer for this important comment. We have clarified the measurement and categorization of decision-making autonomy to ensure transparency and reproducibility. The exposure variable is now described in detail, including its domains, scoring procedures, and the steps used to derive the final binary variable for analysis.

Reviewer Comment 7:

“What influenced the choice of covariates? This is missing in the methods section.”

Author Response (Page 9–10, Line 179–203):

We thank the reviewer for highlighting this important point. The covariates included in our study were selected based on prior evidence and standard DHS analytical practices, reflecting sociodemographic, reproductive, household, and socioeconomic factors known to influence women’s mental health outcomes (Esteban-Gonzalo et al., 2026; M. S. Miah et al., 2026; Tamanna et al., 2025; Zoha et al., 2025). These variables were harmonized with previous DHS-based studies to ensure comparability across analyses and to appropriately control for potential confounding when examining the association between decision-making autonomy and depressive symptoms.

Reviewer Comment 8:

“Line 249: Clarify this confidence interval (AOR = 0.44; 95% CI: 0.271974)”

Author Response (Page 15, Line 324):

We thank the reviewer for pointing this out. We have corrected the formatting of the confidence interval for improved clarity. The revised text now reads: AOR = 0.44 (95% CI: 0.27–0.97), ensuring accurate and reader-friendly interpretation of the reported association.

Reviewer Comment 9:

“Lines 275–279: There is no need repeating the results in the discussion section.”

Author Response (Page 17, Lines 353–358):

We thank the reviewer for this insightful comment. We have revised the discussion section to minimize redundancy and have removed sentences that repeated descriptive results. The discussion now focuses on interpreting the findings, linking them to relevant theoretical frameworks, and emphasizing their implications rather than restating earlier results.

Reviewer Comment 10:

“The discussion section should be anchored in theory.”

Author Response (Page 20, Lines 426–430; Page 19, Lines 395–399):

We thank the reviewer for this valuable comment. In response, we have revised the discussion section to more clearly anchor our interpretation of findings within established theoretical frameworks. Specifically, we have incorporated the Social Determinants of Mental Health framework to explain how structural inequities, gendered power dynamics, and levels of decision-making autonomy influence depressive symptoms among women. These theoretical linkages strengthen the interpretability of the results and provide a more coherent explanation of the observed rural–urban differences.

Reviewer Comment 11:

“What accounted for the spatial variations? This is a key finding of your study and so it needs to be properly discussed.”

Author Response (Page 20, Lines 426–430):

We appreciate the reviewer’s insightful observation. In response, we have expanded the discussion to more fully explain the underlying factors contributing to the observed spatial variations. Specifically, we discuss how geographic disparities reflect differences in women’s decision-making autonomy, socioeconomic inequalities, rural–urban structural conditions, and district-level contextual stressors. These contextual factors help to clarify why certain provinces exhibit higher depressive symptom burdens than others and strengthen the interpretation of our spatial findings.

Reviewer Comment 12:

“Discuss the limitations of using the PHQ-9 in such a study.”

Author Response (Page 21-22, Lines 455–458):

We thank the reviewer for highlighting this important point. We have now added a clear discussion of the limitations associated with the use of the PHQ-9 in this context. Specifically, we note that the PHQ-9 is a self-reported screening tool that may be subject to recall and reporting biases, and that its sensitivity and optimal cut-off points can vary across cultural and population settings. Nevertheless, we emphasize that the PHQ-9 remains one of the most widely validated and commonly used instruments for assessing depressive symptoms in large population-based surveys. These considerations have been incorporated into the revised manuscript.

Reviewer Comment 13:

“The presentation of the discussion is generally problematic. It should address what was known, what this study adds and the implications thereof.”

Author Response (Page 20, Lines 437–439; Page 17–18, Lines 364–382):

We thank the reviewer for this valuable and constructive comment. We have substantially revised the discussion section to improve clarity, structure, and coherence. The revised version now explicitly follows a structured narrative that:

- (i) Summarizes what was previously known regarding depressive symptoms and women’s decision-making autonomy in LMICs, drawing on existing empirical and theoretical literature;
- (ii) Clearly articulates the new contributions of the present study, including the protective role of autonomy, rural–urban heterogeneity, and spatial disparities unique to the Lesotho context; and
- (iii) Outlines the implications of these findings, emphasizing the need for targeted, context-specific mental health and empowerment interventions informed by spatial and sociodemographic patterns.

These revisions make the discussion more concise, theoretically grounded, and aligned with the journal’s expectations for clear interpretation and contribution to the field.

Closing Statement

We sincerely thank **Reviewer 1** for the thoughtful and constructive feedback, which has significantly improved the clarity, rigor, and interpretability of our manuscript. We believe that all

concerns have been carefully addressed, and the revisions have strengthened the study's contribution to understanding the role of women's decision-making autonomy and depressive symptoms in Lesotho. We greatly appreciate your time and insightful comments.

Reviewer 2

Reviewer Comment 1:

Confidence interval formatting – Several confidence intervals appear without appropriate spacing or punctuation, which may confuse readers. For example: "7.40% (95% CI: 6.268.74)" and "51.4758.45". Please revise to standard formats such as "6.26-8.74" or "51.47-58.45".

Response to Reviewer 2:

We sincerely thank the reviewer for this careful observation. We have thoroughly reviewed all confidence intervals throughout the manuscript and corrected their formatting to follow standard conventions (e.g., "6.26–8.74" and "51.47–58.45") to ensure clarity, consistency, and accurate interpretation [[Page 13, Line 272, 273, 276, 278](#)].

Reviewer Comment 2:

Typographical and spacing inconsistencies A few lines contain missing parentheses or spacing errors, particularly in the Results and Table sections. Example: "0.83 0.56-1.23) 0.368". A consistent format such as "0.83 (0.56-1.23), p = 0.368" would improve clarity.

Response to Reviewer 2:

We sincerely thank the reviewer for this careful observation. All typographical and spacing inconsistencies, including parentheses and p-value formatting in the Results and Table 2, have been thoroughly corrected to ensure uniformity, clarity, and improved readability [[Page 42, Line 823–832, Table 2](#)].

Reviewer Comment 3:

Consistency in terminology for depressive symptoms – The manuscript alternates between "moderate-severe," "moderate-to-severe," and "moderate to severe." For example: "A PHQ-9

score of 10 and above was considered a moderate-severe depressive symptom." Please standardize terminology throughout.

Response to Reviewer 2:

We thank the reviewer for this helpful comment. We have carefully standardized the terminology throughout the manuscript to use “**moderate-to-severe**” **depressive symptoms** consistently, ensuring clarity and uniformity across all sections.

Reviewer Comment 4:

Clarification of spatial findings wording – The description of autonomy patterns includes slightly contradictory phrasing: "lower levels of autonomy were centralized towards the southern and south-western provinces... but autonomy was nevertheless poor in general." Consider clarifying whether autonomy is uniformly low or varies meaningfully across regions.

Response to Reviewer 2:

We sincerely thank the reviewer for this valuable suggestion. We have revised the text for clarity as follows:

Updated text:

“Lower levels of autonomy were concentrated in the southern and south-western provinces (Quthing, Qacha’s Nek, Mochale’s Hoek, and Mafeteng), which represent high-burden areas of limited decision-making power. In contrast, the central and northern provinces, including Thaba-Tseka and Maseru, exhibited relatively higher autonomy, although overall empowerment levels remained modest. These geographic patterns highlight meaningful regional variation in female autonomy and identify priority areas for context-specific interventions.”

This revision clarifies that autonomy varies across regions while emphasizing that overall empowerment is still limited [Page 16, Line 331–336].

Reviewer 2 – Comment 5:

Minor improvements to flow and readability

A few sentences in the Results section are long and could be streamlined for clarity. For example, the paragraph describing age patterns, reproductive stressors, and socioeconomic variation could be broken into shorter, more focused sentences to enhance readability.

Response to Reviewer 2:

We thank the reviewer for this helpful suggestion. We have revised the Results section to improve clarity and readability by breaking long sentences into shorter, more focused statements, particularly in the paragraphs describing age patterns, reproductive stressors, and socioeconomic

variations. These edits enhance the flow and make the findings easier to follow without altering their meaning [Page 14, Line 283-292].

Closing Statement

We sincerely thank the reviewer for their careful and constructive feedback. All suggested revisions, including formatting, typographical corrections, clarity in spatial findings, and improvements in discussion and statistical reporting, have been carefully addressed. We believe these changes have significantly enhanced the clarity, readability, and scientific rigor of the manuscript. We greatly appreciate the reviewer's guidance and thoughtful comments, which have helped improve the overall quality of our work.